Knee Examination — OSCE Checklist

Deakin MD2 · MM7 · Time: 8 min (+2 min reading)

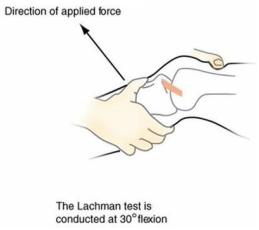
Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* † *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *If fracture suspected: passive movement with great caution, stop if painful or crepitus. Acute loss of circulation or sensation = emergency — don't delay management.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: start standing
☐	Exposure: underwear ideally (shorts for comfort); see the whole limb and quadriceps; compare with the other side throughout
☐	Ask about pain before touching — "Where is it sore?"
☐	General inspection: well or unwell, in pain? Bedside clues (aids, splints)? Vital signs normal?

2. Examination

	Step	What to do / say	Abnormal
Look — standing			
☐	Inspect front, back, sides	Deformity, scars/wounds, quadriceps wasting, swelling/effusion, symmetry	Effusion, previous surgery
☐	Alignment	Valgus / varus; hyperextension	OA (varus common †)
☐	Popliteal fossa	Baker's cyst	OA, RA
☐	Gait	Normal? Antalgic?	
Look — supine			
☐	Re-inspect	Anything missed	
Feel			
☐	Temperature	Compare both knees	Warm = inflammation, infection
☐	Palpate	Patella, joint lines (knee flexed †), tibial tuberosity, collateral ligaments; watch the face	Joint-line tenderness = meniscus
☐	Bulge / sweep test	Knee extended; hand on suprapatellar pouch; sweep medial side up, then lateral side down — watch medial side †	Bulge = small effusion (preferred test)
☐	Patellar tap	Knee extended; hand above patella pushing fluid down; two fingers press patella	Tap = large effusion
Move			
☐	Active flexion / extension	"Bend your knee… straighten it" (0–140° †)	Limited → measure degrees
☐	Passive	Feel for crepitus through flexion and extension	OA, patellofemoral
Special tests — cruciates			
☐	Posterior sag	Both knees flexed to 90°, look from the side	Tibia sags back = PCL tear
☐	Anterior drawer	Knee 90°, sit gently on foot, grasp tibia, pull forward	>6 mm translation = ACL tear
☐	Posterior drawer	As above, push tibia back	Excess backward movement = PCL tear
☐	Lachman's	Knee 20–30°; hold distal thigh, pull proximal tibia forward	Translation / soft end point = ACL



Lachman's test (20–30° flexion)

Special tests — collaterals (knee at 30°)			
☐	MCL	Valgus stress (push knee inwards)	Medial gaping = MCL tear/laxity
☐	LCL	Varus stress (push knee outwards)	Lateral gaping = LCL tear/laxity
Special tests — menisci			
☐	McMurray's — medial	Full flexion; external rotation of tibia + valgus stress, then extend; palpate joint line	Pain, click, locking = medial meniscus
☐	McMurray's — lateral	Full flexion; internal rotation + varus stress, then extend	Pain, click, locking = lateral meniscus
☐	Apley's grind	Prone, knee 90°, stabilise thigh; press down on foot and rotate	Pain or clicking = meniscus
Special tests — patella			
☐	Patellar apprehension	Knee extended; push patella laterally while flexing the knee	Apprehension = patellar instability

3. Closing

☐	Thank patient; help them dress; wash hands
☐	Completion: "I would examine the hip and ankle (joints above and below) and do a neurovascular exam of the lower limb †."
☐	Present: "Normal gait; no deformity, wasting or effusion. No warmth or tenderness. Full range of movement without crepitus. Cruciate, collateral, meniscal and patellar tests were all negative."